جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : فاطمه عبدالرحمن لاشين ~~
المحافظة : حلوان **الحي / المدينة :** حلوان **العنوان :** مساكن صلاح سالم
رقم البطاقة : 25910052200564 **رقم التليفون :** 01226263147 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 05-10-1959 **المهنة :** لاتعمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فاطمه عبدالرحمن لاشين ~~
National ID: 25910052200564



تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Supportive and palliative Treatment**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين

جوستافروسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

صحة مصر

09/09/2026

MDT FORM

Name:	Fatma Abd Elrahman Lashen	**MRN:**	2936
Age:	67 Y	**ID:**	25910052200564
Menopausal Sta	Post	**Phone:***	01100322395
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
Gender			
CO Morbidity:	DM, HTN, Cardiac ISHD 3 stents last 2019, Hepatitis C trea	**Reason Of MDT:**	newly developed bony lesions
Surgical History:*	TAH-BSO> benign , Cholecystectomy , Rt MRM 2020	**ECOG Performance Status**	○ 0 ○ 1 ◉ 2 ○ 3 ○ 4

Clinical Data:

BX: IDC G2

ER: 7/8 **PR:** 7/8 **HER2:** 0 **KI67:** 10%

History:
A case of Right breast cancer since 2020, cT2 N2 luminal A , received neoadjuvant AI (did not receive chemo because of her cardiac condition)> Underwent Rt MRM 10/2020 multicenteric invasive duct carcinoma grade 2 ypT3 N1 (3/28) IHC on surgical pathology ER 6/8 PR 2/8 Her 2 0 Ki 67 7%. Received PORT and on adjuvant Arimidex / Zometa since 2020. C/O of low back pain of 3 months duration, did CT Chest and Abdomen +C 8/2026> Bilateral few small pulmonary nodules seen at both lung parenchyma at lower lobes largest at left lung base measures 5.5 mm likely metastatic - Destructive sclerotic bony lesions at right scapula , Sternum, D11 vertebral body, other ill defined bony lesions scattered at few ribs bilateral with cortical destruction metastatic for bone scan. O/E tenderness at dorsal spine , bilateral hips and left shoulder, no neurological affection

R L



TNM

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:
Metastatic bone , query lung

Pathological Staging:

Sonomammography:

Biomarker/Molecular

Mets Workup:
CT Chest and Abdomen (no pelvis) with contrast 8/2026> Bilateral few small pulmonary nodules seen at both lung parenchyma at lower lobes largest at left lun base measures 5.5 mm likely metastatic - Destructive sclerotic bony lesions at right scapula , Sternum, D11 vertebral body, other ill defined bony lesions scattered at few ribs bilateral with cortical destruction metastatic for bone scan.
No previous CTs done.

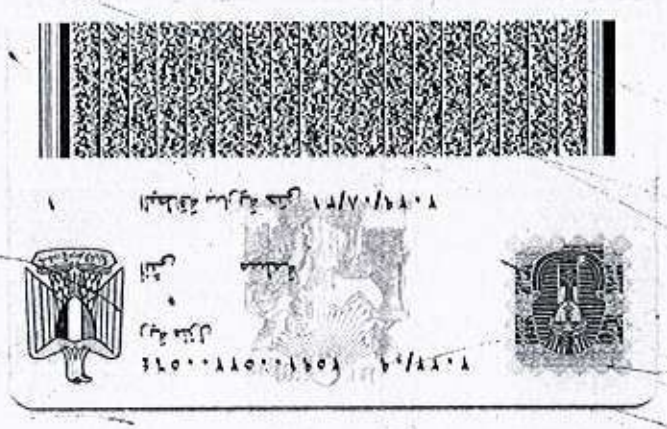
Recommendations:

MDT Outcome & Plan
For CDK 4/6 + Faslodex
Review Bone Scan and Palliative RTH to the indicated areas if needed.
Zometa /3m after dental assessment

SIGNATURE

MDT Form
[[DOCTYPE:MDT_FORM]]





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Name:	**فاطمه عبد الرحمن لاشين**
MR. :	**2936**

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 66-year-old individual who is post-menopausal and has a medical history significant for diabetes mellitus, hypertension, ischemic heart disease with three coronary stents placed in 2019, treated hepatitis C infection, and bilateral knee osteoarthritis. The clinical history notes a right-breast cancer diagnosed in 2020, initially staged clinically as cT2 N2 luminal A. Neoadjuvant aromatase-inhibitor therapy was administered without chemotherapy because of the cardiac condition, followed by a right modified radical mastectomy in October 2020 that revealed a multicentric invasive ductal carcinoma, grade 2, ypT3 N1 (3/28). Post-operative treatment included radiotherapy and adjuvant anastrozole with zoledronic acid (Zometa) since 2020. The patient now reports low back pain of three months' duration.

Pathology from the original biopsy showed invasive ductal carcinoma, grade 2. Immunohistochemistry demonstrated estrogen receptor 7/8, progesterone receptor 7/8, HER2 0, and Ki-67 10 %. Surgical pathology after mastectomy reported ER 6/8, PR 2/8, HER2 0, and Ki-67 7 %. Imaging with contrast-enhanced CT of the chest and abdomen performed in August 2026 identified several small bilateral pulmonary nodules in the lower lobes, the largest measuring 5.5 mm in the left lung base, deemed likely metastatic. Additionally, destructive sclerotic lesions were observed in the right scapula, sternum, D11 vertebral body, and multiple ill-defined rib lesions with cortical destruction, consistent with metastatic bone disease on bone scan. Physical examination revealed tenderness over the dorsal spine, bilateral hips, and left shoulder without neurological deficits.

The multidisciplinary team recommended initiation of a CDK 4/6 inhibitor combined with fulvestrant (Faslodex). A bone scan will be reviewed, and palliative radiotherapy to the indicated skeletal sites will be provided if needed. Continuation of zoledronic acid every three months is planned after a dental assessment.

تحريرا في ,

التاريخ :	**23-09-2026**	**الوقت :**	

الطبيب المعالج :

يعتمد ,

[redacted] Date: 6/2/2020

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Known case of breast cancer.

1. ***Chest:***
   - Right breast mass is seen measuring about 4.2 x 3.5 cm with two enlarged right axillary lymph nodes largest measuring about 2 x 1.3 cm.
   - A single middle lobe pulmonary micronodule is seen measuring 5 mm.
   - No other sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
   - Right hilar calcified lymph node is seen.
   - Normal cardiac size.
   - No pleural or pericardial effusion detected.
   - Normal CT appearance of the thoracic aorta and the great vessels.
2. ***Abdomen and pelvis:***
   - The liver is enlarged in size with homogenous low CT texture. No focal lesions or contour irregularities.
   - Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
   - No intra-hepatic or extra-hepatic biliary radicles dilatation.
   - Normal size & parenchymal texture of the spleen with no focal lesions seen.
   - Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
   - Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
   - Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
   - Normal appearance of the other pelvic organs.
   - No abdominal or pelvic collections.

OPINION:

- ***Right breast mass and pathologically enlarged right axillary lymph nodes.***
- ***Single right pulmonary micro-nodule for follow up.***
- ***Fatty hepatomegaly with no focal lesions.***

CT Scan [[DOCTYPE:CT_SCAN]]

Patient's Name:	**Fatima Abdulrahman Lashin Hassanain**
Date:	**2026-08-25**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Multi-Slice Biphasic Post Contrast CT scan of the Chest and Abdomen with Coronal and 3D Reconstructions Revealed:

- ***Bilateral few small pulmonary nodules seen at both lung parenchyma mainly at lower lobes the largest noted at left lung base measures about (5.5mm) ... Likely metastatic***
- ***Bilateral basal atelectatic bands.***
- *Patent **trachea**, main and segmental bronchi.*
- *Normal both **hilar region**. No hilar masses or lymphadenopathy.*
- *Clear **pleural surfaces** with no evidence of pleural effusion.*
- *Normal **cardiac size**, no pericardial effusion*
- ***Asymmetrically enlarged left thyroid lobe, calcified nodule at right lobe .... For thyroid work up.***

- ***Mild enlarged liver.** No abnormal attenuation values or enhancing lesions.*
- *No dilated intra-hepatic **biliary radicles**.*
- *Normal caliber patent **Portal Vein***
- *Normal size, shape and parenchymal density of the **pancreas**. No enhancing lesions.*
- *Normal size, shape and density of the **spleen**. No focal lesions.*
- *Normal size, and contrast excretion by **both kidneys**. Normal parenchymal thickness. No renal, masses stones or back pressure changes.*
- *Normal both **supra-renal glands**, no masses.*
- *No **ascites***
- *Normal **aorta, I.V.C.***
- *Normal filling of the **urinary bladder** with smooth inner surface and uniform wall thickness.*
- ***Destructive sclerotic bony lesions seen involving the right scapula (glenoid, acromion), Sternum, D11 vertebral body, Another ill defined bony lesions seen scattered at few ribs bilaterally with cortical disruption....Metastatic for bone scan.***





CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Date : 03/09/2024	Patient Name: Fatma Abd Al Rahman Lasheen
Age : 65 Year(s)	Exam : X-RAY CHEST
Pat. ID: 11519	Medical ID: 631/2021

Plain X-ray **Chest** (PA view) revealed:

- *Clear both lung fields.*
- *Clear both costo-phrenic angles.*
- *Border line cardiac size*
- *No bony chest lesion.*

Best Regards,
Dr Dina Gaber, FEBR,

Not Approved

Ultrasound
[[DOCTYPE:ULTRASOUND]]

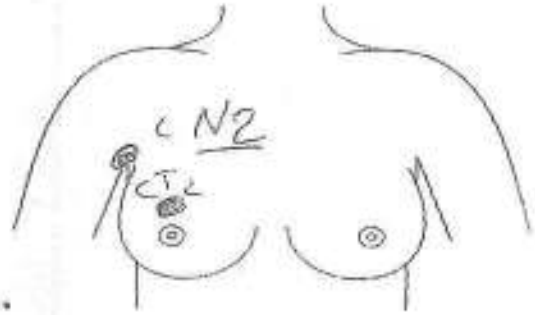
Name: فاطمة عبد الرحمن الشيخ 23/1/2020

Age:

No.: 20

MDT FORM

Clinical data:



.....

.....

.....

.....

Reason of discussion:

.....

..... IDC Luminal A

..... HTN, DM4, IHD post (c) / CT CBA1

MDT Outcome and Plan:

...for Metastatic Work up (Not done) ... bone S

...CT Hcap & Neuro then re MDT.....

...Headwat Chemotherapy after Cycle 4 ...

Signature:

د/ أحمد ياسين

د/ أسماء محمد جلال

د/ [illegible]

MDT Form
[[DOCTYPE:MDT_FORM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Date : 03/09/2024	Patient Name: Fatma Abd Al Rahman Lasheen
Age : 65 Year(s)	Exam : ultrasound ABDOMEN& PELVIS
Pat. ID: 11519	Medical ID: 631/2020

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Enlarged in size (about 17 cm at the right MCL), smooth outline, bright echopattern. No intra hepatic biliary radicle dilatation.*
- *Portal vein is not dilated and patent .*

Gall Bladder: -

- *IS SURGICALLLY REMOVED*
- *CBD is not dilated.*

Spleen: -

- *Average size (About 11 cm), displaying homogenous echopattern.*

Kidneys: -

Are of normal size and site , normal echogenicity , no stone or backpressure changes , no solid or cystic lesion.
-Para-aortic and pancreatic area: no gross lesion

***URINARY BLADDER**: Normal filling. Normal wall thickness, no stones or diverticulae or solid lesion detected.*

***Uterus: -**: is surgically removed*

No free fluid in the Douglas pouch.

Opinion:

Bright hepatomegaly.

Best Regards,
Dr Dina Gaber , FEBR

Not Approved

Ultrasound
[[DOCTYPE:ULTRASOUND]]

Patient name: Fatma Abdel Rahman Lasheen
Date: 23-5-2022

DIGITAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Technique

- left MLO and CC views
- left 3D MLO DBT (Digital Breast tomosynthesis)
- MRM & breast ultrasound

Report:

- Right MRM:
  - Operative bed shows no mass or collection
  - No regional lymphadenopathy.
- Left Breast:
  - ACR B
  - No suspicious micro calcifications or architectural distortion detected.
  - Scattered benign calcificatios.
  - Complementary ultrasound no cystic or solid mass lesions.
  - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
  - Nonspecific axillary LN.

Opinion:

- Post right MRM status showing no evidence of recurrence (BIRADS 2)
- left breast: benign calcifications (BIRADS 2)

Best regards,
Dr. Peter Fahmy

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m7857

اسم المريض	: فاطمه عبدالرحمن لاشين	النوع	: Female
السـن	: 60 Y	العنوان	: القاهرة
تاريخ اخذ العينة	:	تاريخ استلام العينة	: 22/12/2019 12:11
طالب التحليل	: صحة المراة	تاريخ التقرير	: 25/12/2019 13:42
عناية	: صحة المراة		

Primary Sample :

Paraffin Block

Test :

ER &PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor(PR) (IE2)Rabbit monoclonal primary antibody.
Sample recieved was treated by : -VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX.
Sample recived was treated by :- CONFIRM anti Ki67 (30-9) Rabbeit monoclonal primary antibody.

Result :

ER: moderate nuclear staining of > 60 % of tumour cells (core biopsy)
PR: strong nuclear staining of 60 % of tumour cells (core biopsy)
HER2/neu protein over expression : NO membrane staining of tumor cells (core biopsy) .
Ki67 :positive nuclear staining of about 10 % of tumour cells (core biopsy)

Referal Lab :

Comment :

Conclusion :

ER: POSITIVE (SCORE 7 / 8)
PR: POSITIVE (SCORE 7 / 8)
HER2/NEU PROTEIN OVER EXPRESSION : NEGATIVE
Ki67: THE PROLIFERATITION INDEX OF THE TUMOUR IS ABOUT 10 % .

Pathologist: د/تريزا ماهر

Lab Director: د/ تريزا ماهر

19 El-sheikh Rehan Street, Abeideen, Cairo
Tel.(165)7948544 -7950096 Fax. (202)7948544
Mobile . 01144297292
Website:http://www.healthlabs.net
Mail:pathologychl@yahoo.com
Facebook: قسم الباثولوجى - الادارة المركزية للمعامل

This Test Is Not Accredited By EGAC Nor Recognized By ILA

يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها

Immunohistochemistry
[[DOCTYPE:IHC]]

Health Insurance Organizatior
Nasr City Cancer Cente
(NCCC

الهيئة العامة للتامين الصحي
مركز أورام مدينة نصر

اسم المريض : فاطمه عبد الرحمن لاشين

تاريخ الخدمة :5/10/2020

Immunohistochemistry report

Lab-No.:20/608

-Procedure:

-Sections immune stained for ER, PR, Ki67 and Her2neu using supersensitive monoclonal antibodies with Roche envision (Dextran substrate polymer) flex detection system. Pretreatment for heat induced epitope retrieval (HIER) was performed using flex target retrieval solution. Blocking of endogenous peroxidase was performed using Roche peroxidase blocking reagent DAB was used as a chromate.

Results:

ER: (6/8).

PR: (2/8).

Her2: Negative, score = 0

Ki67: 7%

N.B: the slides are delivered to the patient.

Prof.Dr. Hussain Elsayed

Date: 19-Sep-2020 Time: 11:55 am

Immunohistochemistry
[[DOCTYPE:IHC]]

[[DOCTYPE:MDT_FORM]]
MDT Form

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

09/09/2026

MDT FORM

Name:	Fatma Abd Elrahman Lashen	**MRN:**	2936
Age:	67 Y	**ID:**	25910052200564
Menopausal Sta	Post	**Phone:***	01100322395
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
Gender			
CO Morbidity:	DM, HTN, Cardiac ISHD 3 stents last 2019, Hepatitis C trea	**Reason Of MDT:**	newly developed bony lesions
Surgical History:*	TAH-BSO> benign , Cholecystectomy , Rt MRM 2020	**ECOG Performance Status**	○ 0 ○ 1 ◉ 2 ○ 3 ○ 4

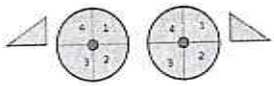
Clinical Data:

BX: IDC G2

ER: 7/8 **PR:** 7/8 **HER2:** 0 **KI67:** 10%

History:
A case of Right breast cancer since 2020, cT2 N2 luminal A , received neoadjuvant AI (did not receive chemo because of her cardiac condition)> Underwent Rt MRM 10/2020 multicenteric invasive duct carcinoma grade 2 ypT3 N1 (3/28) IHC on surgical pathology ER 6/8 PR 2/8 Her 2 0 Ki 67 7%. Received PORT and on adjuvant Arimidex / Zometa since 2020. C/O of low back pain of 3 months duration, did CT Chest and Abdomen +C 8/2026> Bilateral few small pulmonary nodules seen at both lung parenchyma at lower lobes largest at left lung base measures 5.5 mm likely metastatic - Destructive sclerotic bony lesions at right scapula , Sternum, D11 vertebral body, other ill defined bony lesions scattered at few ribs bilateral with cortical destruction metastatic for bone scan.
O/E tenderness at dorsal spine , bilateral hips and left shoulder, no neurological affection

R L



TNM

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:
Metastatic bone , query lung

Pathological Staging:

Sonomammography:

Biomarker/Molecular

Mets Workup:
CT Chest and Abdomen (no pelvis) with contrast 8/2026> Bilateral few small pulmonary nodules seen at both lung parenchyma at lower lobes largest at left lun base measures 5.5 mm likely metastatic - Destructive sclerotic bony lesions at right scapula , Sternum, D11 vertebral body, other ill defined bony lesions scattered at few ribs bilateral with cortical destruction metastatic for bone scan.
No previous CTs done.

Recommendations:

MDT Outcome & Plan
For CDK 4/6 + Faslodex
Review Bone Scan and Palliative RTH to the indicated areas if needed.
Zometa /3m after dental assessment

SIGNATURE

[redacted] Date: 6/7/2020

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Known case of breast cancer.

1. ***Chest:***
   - Right breast mass is seen measuring about 4.2 x 3.5 cm with two enlarged right axillary lymph nodes largest measuring about 2 x 1.3 cm.
   - A single middle lobe pulmonary micronodule is seen measuring 5 mm.
   - No other sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
   - Right hilar calcified lymph node is seen.
   - Normal cardiac size.
   - No pleural or pericardial effusion detected.
   - Normal CT appearance of the thoracic aorta and the great vessels.
2. ***Abdomen and pelvis:***
   - The liver is enlarged in size with homogenous low CT texture. No focal lesions or contour irregularities.
   - Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
   - No intra-hepatic or extra-hepatic biliary radicles dilatation.
   - Normal size & parenchymal texture of the spleen with no focal lesions seen.
   - Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
   - Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
   - Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
   - Normal appearance of the other pelvic organs.
   - No abdominal or pelvic collections.

OPINION:

- ***Right breast mass and pathologically enlarged right axillary lymph nodes.***
- ***Single right pulmonary micro-nodule for follow up.***
- ***Fatty hepatomegaly with no focal lesions.***

CT Scan [[DOCTYPE:CT_SCAN]]

Patient's Name:	**Fatima Abdulrahman Lashin Hassanain**
Date:	**2026-08-25**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Multi-Slice Biphasic Post Contrast CT scan of the Chest and Abdomen with Coronal and 3D Reconstructions Revealed:

- ***Bilateral few small pulmonary nodules seen at both lung parenchyma mainly at lower lobes the largest noted at left lung base measures about (5.5mm) ... Likely metastatic***
- ***Bilateral basal atelectatic bands.***
- *Patent **trachea**, main and segmental bronchi.*
- *Normal both **hilar region**. No hilar masses or lymphadenopathy.*
- *Clear **pleural surfaces** with no evidence of pleural effusion.*
- *Normal **cardiac size**, no pericardial effusion*
- ***Asymmetrically enlarged left thyroid lobe, calcified nodule at right lobe .... For thyroid work up.***

- ***Mild enlarged liver.** No abnormal attenuation values or enhancing lesions.*
- *No dilated intra-hepatic **biliary radicles**.*
- *Normal caliber patent **Portal Vein***
- *Normal size, shape and parenchymal density of the **pancreas**. No enhancing lesions.*
- *Normal size, shape and density of the **spleen**. No focal lesions.*
- *Normal size, and contrast excretion by **both kidneys**. Normal parenchymal thickness. No renal, masses stones or back pressure changes.*
- *Normal both **supra-renal glands**, no masses.*
- *No **ascites***
- *Normal **aorta, I.V.C.***
- *Normal filling of the **urinary bladder** with smooth inner surface and uniform wall thickness.*
- ***Destructive sclerotic bony lesions seen involving the right scapula (glenoid, acromion), Sternum, D11 vertebral body, Another ill defined bony lesions seen scattered at few ribs bilaterally with cortical disruption....Metastatic for bone scan.***





CT Scan
[[DOCTYPE:CT_SCAN]]

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصري لصحة مصر

09/09/2026

MDT FORM

Name:	Fatma Abd Elrahman Lashen	**MRN:**	2936
Age:	67 Y	**ID:**	25910052200564
Menopausal Sta	Post	**Phone:***	01100322395
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
Gender			
CO Morbidity:	DM, HTN, Cardiac ISHD 3 stents last 2019, Hepatitis C trea	**Reason Of MDT:**	newly developed bony lesions
Surgical History:*	TAH-BSO> benign , Cholecystectomy , Rt MRM 2020	**ECOG Performance Status**	○ 0 ○ 1 ◉ 2 ○ 3 ○ 4

Clinical Data:

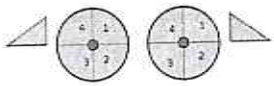
BX: IDC G2

ER: 7/8 **PR:** 7/8 **HER2:** 0 **KI67:** 10%

History:

A case of Right breast cancer since 2020, cT2 N2 luminal A , received neoadjuvant AI (did not receive chemo because of her cardiac condition)> Underwent Rt MRM 10/2020 multicenteric invasive duct carcinoma grade 2 ypT3 N1 (3/28) IHC on surgical pathology ER 6/8 PR 2/8 Her 2 0 Ki 67 7%. Received PORT and on adjuvant Arimidex / Zometa since 2020. C/O of low back pain of 3 months duration, did CT Chest and Abdomen +C 8/2026> Bilateral few small pulmonary nodules seen at both lung parenchyma at lower lobes largest at left lung base measures 5.5 mm likely metastatic - Destructive sclerotic bony lesions at right scapula , Sternum, D11 vertebral body, other ill defined bony lesions scattered at few ribs bilateral with cortical destruction metastatic for bone scan.
O/E tenderness at dorsal spine , bilateral hips and left shoulder, no neurological affection

R L



TNM

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:
Metastatic bone , query lung

Pathological Staging:

Sonomammography:

Biomarker/Molecular

Mets Workup:

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Recommendations:

MDT Outcome & Plan

For CDK 4/6 + Faslodex
Review Bone Scan and Palliative RTH to the indicated areas if needed.
Zometa /3m after dental assessment

SIGNATURE

[redacted]

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Known case of breast cancer.

1. ***Chest:***
   - Right breast mass is seen measuring about 4.2 x 3.5 cm with two enlarged right axillary lymph nodes largest measuring about 2 x 1.3 cm.
   - A single middle lobe pulmonary micronodule is seen measuring 5 mm.
   - No other sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
   - Right hilar calcified lymph node is seen.
   - Normal cardiac size.
   - No pleural or pericardial effusion detected.
   - Normal CT appearance of the thoracic aorta and the great vessels.
2. ***Abdomen and pelvis:***
   - The liver is enlarged in size with homogenous low CT texture. No focal lesions or contour irregularities.
   - Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
   - No intra-hepatic or extra-hepatic biliary radicles dilatation.
   - Normal size & parenchymal texture of the spleen with no focal lesions seen.
   - Normal size, parenchymal thickness & density of both kidneys with no stones or back pressure changes.
   - Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
   - Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
   - Normal appearance of the other pelvic organs.
   - No abdominal or pelvic collections.

OPINION:

- ***Right breast mass and pathologically enlarged right axillary lymph nodes.***
- ***Single right pulmonary micro-nodule for follow up.***
- ***Fatty hepatomegaly with no focal lesions.***

Patient's Name:	**Fatima Abdulrahman Lashin Hassanain**
Date:	**2026-08-25**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Multi-Slice Biphasic Post Contrast CT scan of the Chest and Abdomen with Coronal and 3D Reconstructions Revealed:

- ***Bilateral few small pulmonary nodules seen at both lung parenchyma mainly at lower lobes the largest noted at left lung base measures about (5.5mm) ... Likely metastatic***
- ***Bilateral basal atelectatic bands.***
- *Patent **trachea**, main and segmental bronchi.*
- *Normal both **hilar region**. No hilar masses or lymphadenopathy.*
- *Clear **pleural surfaces** with no evidence of pleural effusion.*
- *Normal **cardiac size**, no pericardial effusion*
- ***Asymmetrically enlarged left thyroid lobe, calcified nodule at right lobe .... For thyroid work up.***

- ***Mild enlarged liver.** No abnormal attenuation values or enhancing lesions.*
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- *Normal caliber patent **Portal Vein***
- *Normal size, shape and parenchymal density of the **pancreas**. No enhancing lesions.*
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- *No **ascites***
- *Normal **aorta, I.V.C.***
- *Normal filling of the **urinary bladder** with smooth inner surface and uniform wall thickness.*
- ***Destructive sclerotic bony lesions seen involving the right scapula (glenoid, acromion), Sternum, D11 vertebral body, Another ill defined bony lesions seen scattered at few ribs bilaterally with cortical disruption....Metastatic for bone scan.***





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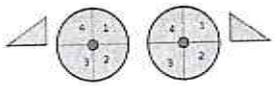
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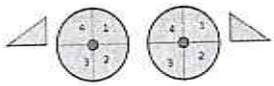
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NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Fatima Abdulrahman Lashin	**Date : 06-10-2026 06:41**
Branch : Maadi	**Medical No. : 477460**
Ref.Physician Prof : Goustav Roussey Egypt	**Ins.Company : التامين الصحي**

Tc-99m MDP WHOLE-BODY BONE SCINTIGRAPHY REPORT

Clinical Indication: Staging and follow-up for skeletal metastatic disease in a 67-year-old female with right breast carcinoma, post right mastectomy and radiotherapy, on hormonal therapy, with known severe osteoarthritis, presenting with bone pain.

Procedure: Whole-body anterior and posterior planar bone scintigrams were acquired following intravenous Tc-99m MDP administration.

FINDINGS

- Multiple intense focal hyperconcentrations are seen throughout the dorsal and lumbar vertebrae, with focal accumulation in the sternum and the posterior arcs of the left ribs.
- Multiple focal areas of abnormal uptake involve the pelvic bones, most prominent at the right sacroiliac region and right acetabulum, with normal physiological distribution in the calvarium.
- Marked intense focal hyperconcentration is seen in the right shoulder girdle, with focal uptake also in the left scapula.
- Increased articular uptake is noted at both knees, consistent with known severe degenerative osteoarthritis, without focal cortical diaphyseal lesions in the long bones.
- Preserved renal excretion with normal bladder accumulation and no extraskeletal localization.

IMPRESSION

- Widespread multifocal avid osteoblastic osseous lesions, highly suspicious for active skeletal metastases in the setting of known right breast carcinoma, for radiological correlation and clinical oncology evaluation.

Dr. Alaa Nagi, MD

Nuclear Medicine

NILE DIAGNOSTIC CENTER

Call Center 19656

Bone Scan
[[DOCTYPE:BONE_SCAN]]